

Who owns your doctor? A decade of trends in physician practice ownership in dermatology and gastroenterology

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Abstract

Introduction How physicians deliver care is shaped by who owns their practice, yet no study comprehensively maps the ownership landscape within a specialty.

Methods Using Medicare claims and hand-collected transaction data from 2012 to 2022, we classified dermatologists and gastroenterologists into 5 mutually exclusive ownership categories—independent, corporate chain, hospital or health system, insurer, and supergroup—and distinguished 2 pathways out of independent practice: acquisition and organic physician entry.

Results Independent practice declined from 62% to 44% of dermatologists and from 52% to 31% of gastroenterologists. By 2022, corporate chains, nearly all private-equity backed, accounted for 16% of dermatologists and 11% of gastroenterologists, while hospital ownership reached 26% and 47%. Hospitals were the largest nonindependent owners in both specialties; corporate chains grew fastest in dermatology. Corporate chains pursued different growth strategies by specialty: Acquisitions of established physician-owned supergroups dominated gastroenterology, while growth in dermatology also came from building new chains from small practices. Increasingly, corporate growth was driven by organic entry of both experienced physicians and recent graduates, overtaking acquisitions by 2021 in dermatology.

Conclusion Who owns a doctor's practice, and how that ownership grows, varies widely—heterogeneity that has implications for patient care and policy addressing provider consolidation.

Keywords: healthcare markets, organization of medicine, consolidation, mergers and acquisitions, dermatology, gastroenterology

Key takeaways

- The shift away from independent practice occurs through different owners in different specialties—corporate chains in dermatology, hospitals in gastroenterology—so studies that track one owner type or pool them together may miss the larger picture.
- Large physician-owned “supergroups” were more likely than smaller independent practices to be acquired, frequently serving as a stepping stone to corporate ownership.
- Corporate chains increasingly grew by recruiting individual physicians rather than by acquiring

practices, a channel that measures of consolidation built around transactions do not capture.

Introduction

The organizational environment in which a physician practices medicine can shape how they deliver care.^{1,2} For example, changes in practice ownership can influence prices, volume, and quality of services that physicians provide^{3–15} and have been the subject of substantial regulatory interest.¹⁶ Practice mergers and acquisitions and voluntary physician movement into larger organizations have transformed the practice ownership landscape, with the share of physicians in independent practice falling from 60% to 47% between 2012 and 2022.¹⁷

Despite these consequential changes, no study has systematically documented practice ownership trends across the full

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range of entities acquiring physician practices within a specialty. Challenges in collecting comprehensive ownership data have contributed to the literature developing largely in parallel silos, with distinct bodies of work on specific owner types such as private equity (PE) or hospitals. Industry reports seeking to characterize ownership trends more broadly have relied on self-reported physician surveys.^{17,18} More recent work has begun to map this broader landscape by focusing on the flow of ownership change, identifying group-to-group mergers, and differentiating health system from nonhealth system acquirers, but observes consolidation only through discrete transactions—missing ownership growth that occurs without a merger—and does not classify the full set of practices into mutually exclusive owner-type categories.¹⁹ Another maps the stock of practices but generates broad ownership categories that collapse insurers, retail companies, and large physician-owned groups into a single corporate category, applying a parent-size threshold that can misclassify smaller corporate-owned practices as independent and larger physician-owned groups as corporate.²⁰

In this paper, we extend and combine the methods and classification of this work by documenting important sources of heterogeneity in the ownership landscape, both across the types of organizations acquiring practices (eg, corporate chain, hospital, insurer) and within ownership categories, that may have distinct implications for physician behavior. For example, within the broader category of “corporate” ownership, a PE firm that creates its own chain organization to build and buy practices may exert a different level of control over practice operations and clinical decision-making than one that acquires an already-large, physician-built group with an established brand (which we refer to as “supergroups”),²¹⁻²³ These physician-owned “supergroups” have received relatively little attention in the literature, but also acquire practices and operate at a scale that may confer many of the same administrative and operational advantages as corporate chains or hospital systems.

In this study, we provide the first comprehensive claims-based characterization of the changing ownership landscape in dermatology and gastroenterology from 2012 to 2022. We classify practices into 5 mutually exclusive ownership categories—*independent*, *corporate chain*, *hospital/health system* (“*hospital*”), *insurer*, and *supergroup*—and document the distribution of physicians across them over this period. Across these categories, owners share a financial interest in capturing value from the practices they own, but a hospital, an insurer, and a corporate chain capture that value through different mechanisms and can exert different degrees of control over their member physicians. Documenting how physicians are distributed across these categories, and how that distribution has changed over time, is a necessary first step toward understanding how ownership shapes physician behavior and patient care, the broader question this descriptive study is intended to support.

We focus on dermatology and gastroenterology because they have been the subject of a large body of prior work on physician practice ownership, have experienced substantial external investment, and span both office and facility-based procedures.^{3,24,4-7,25,26,19} Together, these specialties account for a substantial volume of care: Dermatologists conduct an estimated 44 million office visits per year,²⁷ and gastroenterologists perform over 15 million colonoscopies annually.²⁸

Our study makes 4 main contributions. First, this is the first taxonomy to span the full set of acquirer types within a specialty,

assigning practices into mutually exclusive categories rather than tracking a single owner type. Second, within the corporate chain category, we distinguish 3 archetypes reflecting different firm strategies. Third, we extend prior methodologies for identifying practice ownership changes, which have relied on either algorithmically detected acquisitions or hand-collected transaction data, by combining both in a single approach. Fourth, we separately characterize acquisition-driven and organic physician entry as distinct pathways into corporate chains.

We document substantial declines in independent practice in both specialties, driven by growth in corporate and hospital ownership. We also document the emergence of physician-owned supergroups, many ultimately acquired by corporate chains and hospitals, and show that corporate growth increasingly occurs through voluntary physician entry rather than acquisition alone. These findings highlight the importance of moving beyond single-category analyses when evaluating the consequences of ownership change.

Study data and methods

Data and sample construction

We identified physicians with a specialty of dermatology or gastroenterology using the Centers for Medicare & Medicaid Services’ Medicare Data on Provider Practice and Specialty (MD-PPAS) files, which contain records for all providers who bill Medicare (Appendix 1.1 provides details on sample construction). The final analytic sample consisted of 16 320 dermatologists across 8151 unique practices and 18 958 gastroenterologists across 6938 unique practices.

Medicare Data on Provider Practice and Specialty also records the 2 tax identification numbers (TINs) under which a provider, identified by their national provider identifier (NPI), most frequently billed Medicare each month. Prior studies have used TINs to proxy for physician practices,^{2,29-31} and NPI transitions between TINs to flag potential acquisitions.^{6,8,15,19} We adapted this approach by first assigning each physician to a single “main” TIN per month using a sequential tiebreak system based on billing volume and then applying smoothing procedures to distinguish true practice changes from temporary billing artifacts (Appendix 2). We then flagged potential acquisitions when at least half of physicians exited an “origin” TIN within a given month, identifying the “destination” TIN as the practice receiving the plurality of departing physicians in the 3 months following the exit (Appendix 3.1).

Since TINs may not change postacquisition or may change for reasons unrelated to an acquisition (eg, group rebranding), we manually verified algorithmically identified ownership changes and supplemented them with additional acquisition events identified using the sources described below (Appendix 3.2). Through this process, we categorized each TIN into 5 mutually exclusive ownership categories at the TIN-year level: *corporate chain*, *hospital*, *insurer*, *supergroup*, and *independent*.

We defined a “corporate chain” as a multiunit firm, backed by a financial sponsor (eg, PE firm, venture capital [VC] firm) or operated by a physician management company, that acquires medical practices and delivers similar services across units under centralized management. We identified corporate chains using PitchBook, industry reports, and lists published in previous research^{26,32-34,6} and hand-collected acquisition events using

PitchBook, press releases, and internet searches. See [Appendix 4](#) for details on these entities. Our hand-collected approach identified 125 (16%) additional acquisitions that were not flagged by the algorithm, illustrating the advantage of the 2-pronged approach. We further distinguished 3 corporate chain archetypes representing different firm strategies: large group practices that were acquiring other practices before receiving financial backing (specifically from PE firms) (“Type 1”), small regional practices that began acquiring only after receiving financial backing (either PE or VC funding) (“Type 2”), and management companies or chains created de novo by nonphysician business executives or a financial firm (either PE or VC funding) (“Type 3”). We utilize an inclusive definition of corporate chain to ensure all practices are categorized, though in practice, the majority of corporate-owned physicians are in PE-backed chains (96% in dermatology and 97% in gastroenterology). See [Appendix Figure 2](#) for a decomposition of corporate chain ownership by PE, VC, and management companies and [Appendix 4](#) for more details on this categorization.

To identify hospital-owned practices, we matched TIN names in MD-PPAS to known hospital and health system names and used algorithmically identified TIN switches in which the destination TIN was associated with a hospital.³⁵ To capture ownership changes not associated with a TIN switch, we supplemented this approach with changes in health system affiliation recorded in the Agency for Healthcare Research and Quality Compendium of US Health Systems,^{19,36,37} combined with manual searches to verify an acquisition took place ([Appendix 3.2.4](#)). To identify insurer-owned practices, we used a similar approach ([Appendix 3.2.3](#)), matching TIN names to known insurers and supplementing with manual searches, existing literature,¹⁰ and a list of UnitedHealth’s largest practice acquisitions.³⁸

Following the legal and practitioner literature, we adopted the term “supergroup” to represent a distinct organizational form of independent, physician-owned practices that consolidate under a single TIN to “realize cost savings and efficiencies of scale” under unified management.²¹ Prior research has discussed similar groups in primary care using size-based criteria.^{30,39,40} Adopting this approach, we defined supergroups as TINs that ever reached the 95th percentile of the specialty-specific practice size distribution (at least 7 dermatologists or 13 gastroenterologists) and that were not insurer-, hospital-, or corporate-owned in a given year. We selected the 95th percentile because it situated these groups at approximately the average size of hospital-, corporate-, and insurer-owned practices ([Appendix Table 3](#)), thereby identifying physician-owned groups that had reached a scale comparable to externally owned organizations. Additional details on supergroups, and the ways in which they differ from other independent groups, are provided in [Appendix 5](#). The remaining physician-owned practices—those that never reached this threshold—were designated as independent.

This study was approved by the University of California, Berkeley Institutional Review Board Protocol Number 2024-2003-17272.

Limitations

This study has several limitations. First, our sample is limited to physicians who bill Medicare Part B fee-for-service claims and

therefore does not capture physicians who exclusively serve commercially insured or nonelderly populations or perform only cosmetic procedures. However, the majority of dermatologists and gastroenterologists participate in Medicare, with fewer than 2% of physicians in either specialty formally opting out,⁴¹ and nearly all corporate chains identified through PitchBook and web searches also appeared in MD-PPAS (with few exceptions; see [Appendix 1.2](#)). Because excluded practices are, if anything, more likely to be corporate-owned, this restriction understates corporate ownership and overstates independent practice. Second, a TIN is an imperfect proxy for a physician’s practice, as it represents a billing relationship rather than a clinical entity.^{29,42} The direction of the bias is ambiguous: A single TIN may encompass multiple practice sites or a single practice may bill under multiple TINs. However, this approach is consistent with prior studies.^{2,19,43} Third, hand-collected data are subject to incompleteness or inaccuracy. We mitigate these concerns by cross-validating hand-collected data against algorithmically flagged events. Fourth, our classification reflects ownership of physician practices but gastroenterologists may perform procedures in ambulatory surgery centers (ASCs) that are wholly or partially owned by PE.⁴⁴ To the extent that some physicians we classify as independent operate within their ASCs, the effective scope of corporate influence may exceed what our practice-level estimates suggest. Finally, while our supergroup definition is empirically anchored to the size of externally owned practices, we show in [Appendix 5.3](#) that our results are qualitatively unchanged across alternative percentile thresholds.

Study results

Trends in ownership of physicians between 2012 and 2022

Practice consolidation increased substantially in both specialties. In dermatology, the total number of physicians grew by 18%, while the number of unique practices declined by 19%, a pattern consistent with consolidation into larger organizations ([Table 1](#)). Among physicians, the share in independent practice fell from 62% in 2012 to 44% in 2022; among practices, the independent share fell from 88% to 82%. Corporate chains exhibited the largest growth of any ownership category, increasing from 1% in 2012 to 16% of dermatologists by 2022, though they represented only 5% of total practices in 2022. Hospital ownership grew from 20% to 26% of physicians and was the largest nonindependent category throughout the period. Insurer ownership was negligible throughout. The share of physicians in supergroups declined from 17% to 13% as supergroups were acquired by corporate chains, hospitals, and insurers.

In gastroenterology, a similar pattern held: The physician workforce grew by 12%, while the number of unique practices shrank by 29% ([Table 1](#)). The share of physicians in independent practices declined from 52% in 2012 to 31% in 2022, while the share of independent practices fell from 83% to 72%. The share of physicians in corporate chains increased by 11 percentage points (from <1% to 11%), while the share in hospital-owned practices increased by 14 percentage points (from 33%

Table 1. Changes in counts of physicians and practices by ownership type, 2012 vs 2022.

	Unique physician count				Unique practice count			
	2012 (%)	2022 (%)	Difference (2022-2012)		2012 (%)	2022 (%)	Difference (2022-2012)	
			Count	Percentage point			Count	Percentage point
Dermatology								
Independent	6835 (62%)	5788 (44%)	−1047	−18	4922 (88%)	3700 (82%)	−1222	−6
Corporate	113 (1.0%)	2146 (16%)	+2033	+15	20 (0.4%)	206 (4.6%)	+186	+4.2
Hospital/health system	2258 (20%)	3353 (26%)	+1095	+6	352 (6.3%)	411 (9.1%)	+59	+2.8
Insurer	3 (0.0%)	83 (0.6%)	+80	+0.6	1 (0.0%)	8 (0.2%)	+7	+0.2
Supergroup	1874 (17%)	1738 (13%)	−136	−4	292 (5.2%)	202 (4.5%)	−90	−0.7
Total	11 083	13 108	+2025		5587	4527	−1060	
Gastroenterology								
Independent	7173 (52%)	4715 (31%)	−2458	−21	3812 (83%)	2357 (72%)	−1455	−11
Corporate	51 (0.4%)	1734 (11%)	+1683	+10.6	1 (0.0%)	62 (1.9%)	+61	+1.9
Hospital/health system	4478 (33%)	7233 (47%)	+2755	+14	645 (14%)	763 (23%)	+118	+9
Insurer	8 (0.1%)	109 (0.7%)	+101	+0.6	2 (0.0%)	10 (0.3%)	+8	+0.3
Supergroup	1985 (14%)	1497 (9.8%)	−488	−4.2	116 (2.5%)	72 (2.2%)	−44	−0.3
Total	13 695	15 288	+1593		4576	3264	−1312	

The column “2022-2012” reports the row differences in counts and percentage points between 2022 and 2012. Each physician is assigned to the ownership type from their last observed month in a given year. Each practice (tax identification number) is assigned the modal ownership type of its physicians in December of a given year. Totals include physicians/practices classified as “multiple” (overlapping ownership flags); these are omitted as a separate row due to negligible counts. Source: Authors’ analysis of data from the Medicare Data on Provider Practice and Specialty, PitchBook, Agency for Healthcare Research and Quality Compendium of US Health Systems, press releases, other publications, and internet searches.

to 47%), overtaking independent practice as the single largest ownership category. Insurer ownership remained negligible, and the share of physicians in supergroups declined from 14% to 10%.

Figure 1 reveals important temporal dynamics within these trends. Among dermatologists, corporate ownership exhibited notable growth beginning early in the study period, while hospital ownership remained relatively stable (Figure 1a). By contrast, hospital ownership dominated the nonindependent landscape among gastroenterologists across the study period, with corporate ownership not emerging as a growing sector until 2017 (Figure 1b). For both specialties, supergroups became an increasingly smaller share of the ownership landscape, with this decline more pronounced among gastroenterologists and coinciding with the rise of corporate ownership.

Of 8151 unique dermatology practices observed during our study period, 1001 (12%) experienced ownership changes (Appendix 6, Appendix Table 7), with these events peaking in 2017-2018 (Figure 2a; Appendix Figure 3A). The majority (77%) of these transactions were driven by corporate chains (Appendix Table 7). Of 6938 unique gastroenterology practices, 850 (12%) experienced ownership changes, with these events peaking in 2020-2021 (Figure 2b; Appendix Table 7; Appendix Figure 3B). The composition of these transactions was split into 2 distinct periods: Hospitals dominated early ownership changes (2012-2017, accounting for 71% of total transactions during this period), giving way to rapid growth in corporate activity in the later period (2018-2022, 45% of total transactions during this period) (Appendix Table 7).

Corporate subtypes and supergroups

We identified 43 unique corporate chains in dermatology and 11 in gastroenterology (Appendix Table 8). Type 1 chains were the dominant corporate archetype in gastroenterology, accounting for 83% of physicians by 2022. Notably, there were zero type 1 (or type 2) gastroenterology practices in 2012. In dermatology, type 1 and type 3 chains were more evenly distributed, accounting for 49% and 44% of physicians, respectively, by 2022.

A significant fraction of supergroups were themselves acquired during the study period. In dermatology, of 325 unique supergroup practices, 32% experienced at least one ownership change; in gastroenterology, of 143 unique supergroup practices, nearly 43% experienced at least one ownership change. Corporate chains were the dominant acquirer of supergroups in both cases (30% and 34% in dermatology and gastroenterology, respectively) (Appendix Table 4). In contrast, only around 11% of nonsupergroup independent practices were acquired during the study period (Appendix Table 5).

Organic vs acquisition-driven growth

We classified 2 distinct pathways of corporate growth: acquisitions and organic entry (Figure 3). We defined acquisition-driven entry as physicians whose practice was acquired by a corporate chain and organic entry as physicians who either transitioned from a noncorporate to a corporate practice without a corresponding acquisition event or who first appeared in our data within a corporate-owned practice. In dermatology, the relative contribution of these 2 pathways shifted considerably

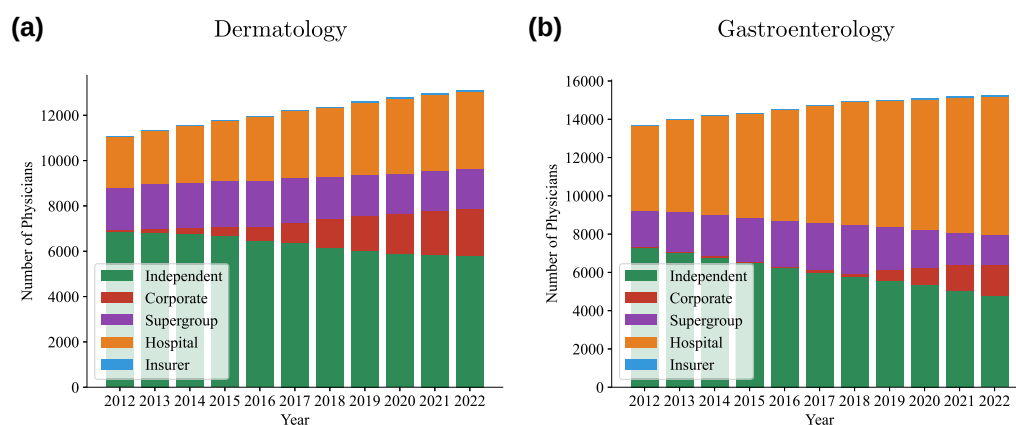


Figure 1. Trends in physician practice ownership, 2012-2022. Each bar represents the number of physicians under each ownership category by year. Each physician is assigned to the ownership type from their last observed month in a given year. Source: Authors' analysis of data from the Medicare Data on Provider Practice and Specialty, PitchBook, Agency for Healthcare Research and Quality Compendium of US Health Systems, press releases, other publications, and internet searches.

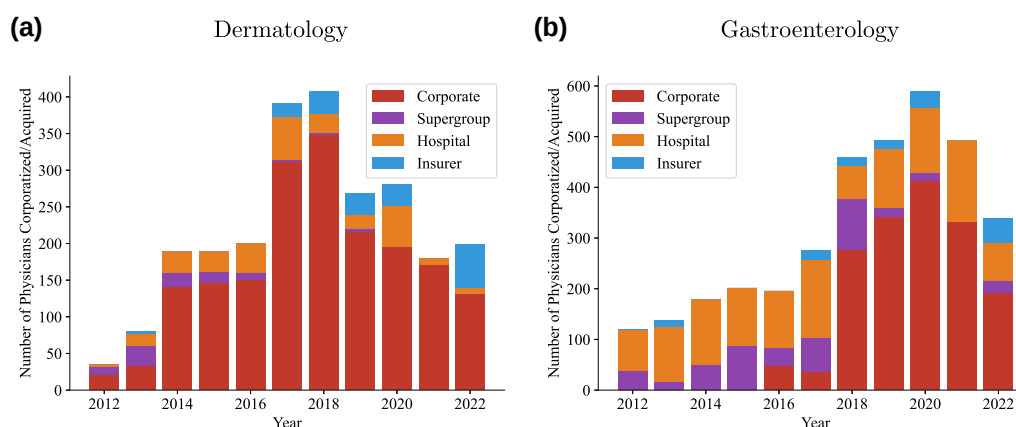


Figure 2. Number of physicians experiencing an ownership change, 2012-2022. Each bar represents the number of physicians impacted by an ownership change to a corporate chain, supergroup, hospital/health system, or insurer. Source: Authors' analysis of data from the Medicare Data on Provider Practice and Specialty, PitchBook, Agency for Healthcare Research and Quality Compendium of US Health Systems, press releases, other publications, and internet searches.

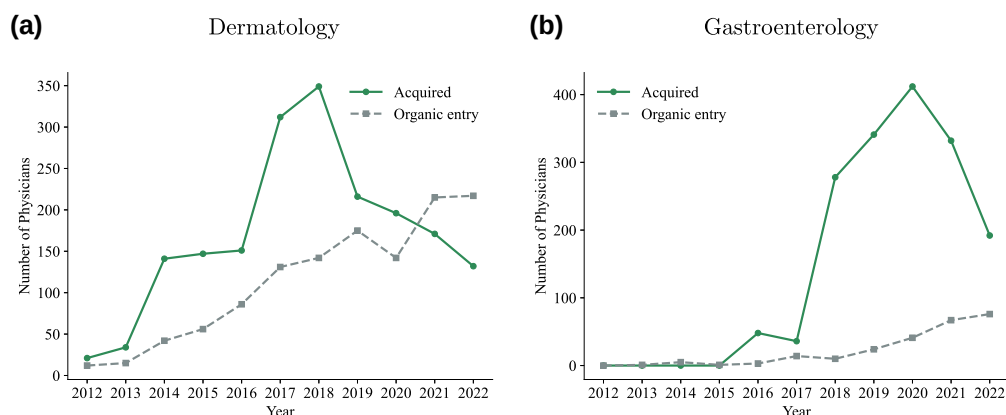


Figure 3. Drivers of corporate chain growth: organic entry vs acquisition, 2012-2022. Each line represents the annual count of physicians entering corporate-owned practices through 1 of 2 channels. Acquired physicians are those whose practice was acquired by a corporate chain. Organic entry physicians are those who appear in a corporate-owned practice without a corresponding acquisition event, comprising of voluntary moves and new-to-data entries. Source: Authors' analysis of data from the Medicare Data on Provider Practice and Specialty, PitchBook, Agency for Healthcare Research and Quality Compendium of US Health Systems, press releases, other publications, and internet searches.

(Figure 3a). Acquisition-driven entry increased rapidly from 2012 to 2018, but declined markedly beginning in 2019; by 2021–2022, organic entry outpaced acquisitions for the first time. Among gastroenterologists, acquisition-driven entry rapidly increased in 2018 and remained dominant through the rest of the study period (Figure 3b). However, as in dermatology, acquisition-driven entry began declining after 2020, while organic growth continued to increase. Appendix Figure 4 further decomposes organic entry into “incumbent” switchers who practiced in another noncorporate group and “newcomers” who are mostly recent residency graduates. Organic growth in dermatology was driven roughly equally across these 2 groups (with incumbent switchers slightly larger), while incumbent switchers outpaced newcomer entrants starting in 2020 in gastroenterology.

Discussion

Both dermatologists and gastroenterologists shifted substantially away from independent practice over our study period. In dermatology, corporate entities grew early and became the second-largest nonindependent category by 2022 (16%) behind hospital ownership (26%); this growth was driven primarily by acquisitions, though organic entry overtook acquisition as the dominant channel in 2021. In gastroenterology, hospital ownership was dominant (47%), while corporate ownership emerged later and remained small but growing through the end of the study period (11%), also driven primarily by acquisition, though with a rising share of organic entry. Supergroups declined in both specialties, from 17% to 13% in dermatology and 14% to 10% in gastroenterology, as over a third were acquired by hospitals and corporate chains. We find minimal insurer ownership during this period.

Understanding these trends by owner type matters because owners acquire practices for different reasons, with implications for patients and for policy. Hospitals and health systems may acquire physician practices to exploit payment differentials across sites of care, obtain referrals, or increase bargaining leverage with insurers.^{12,13,15} Insurers may acquire physician practices to control the total cost of care, foreclose rivals, upcode, or game the medical loss ratio.^{10,11,45} Corporate chains may acquire or build practices to maximize returns for a financial sponsor through cost-cutting measures and enhancing productivity, as well as increasing bargaining leverage with insurers.^{3,8,46,47} Supergroups may band together into large group practices as a means of competing with insurer-, hospital-, and corporate-owned groups. Research has generally shown that both horizontal and vertical practice consolidation lead to higher prices,^{4,6,12,15,29,43} but changes to access, procedures, treatment, and quality are often mixed or null and vary across specialties.^{5,6,44,48} To our knowledge, no single paper has examined how these key outcomes differ across acquirer types within a single specialty, an area of future research that our study seeks to support.

These differing motives also relate to current policy debates over how to curb consolidation.^{49–51} For example, Medicare’s incremental move toward site-neutral payment seeks to equalize reimbursement for the same service across settings. By removing the hospital facility-fee premium, it weakens a key financial incentive for hospitals to acquire practices.^{49,51} Corporate chain incentives, by contrast, are left intact and may even strengthen in relative terms: Equalizing payment across sites of care reduces

the price advantage that hospital outpatient departments held over corporate office- or ambulatory surgical center (ASC)-based practices and potentially improves corporate chains’ relative position as acquirers.⁵² Because site-neutral payment does not ease the financial and administrative pressures that lead independent physicians to sell or discipline acquisition activity directly, it may accelerate the shift of consolidation toward corporate chains.

In addition to heterogeneity across ownership types, our study documents heterogeneity within corporate chains. By 2022, type 1 chains, practices that were already large supergroups when they obtained financial backing, dominated in gastroenterology (83% of corporate-owned physicians), whereas in dermatology, they were comparable in share to type 3 chains, created *de novo* by corporate entities (49% and 44%, respectively). The prevalence of type 3 chains in dermatology suggests its fragmented landscape of small practices⁵³ created opportunities to build new chains, whereas in gastroenterology, corporate entities more often acquired supergroups that had already achieved scale. This within-chain heterogeneity may help explain null and mixed findings of corporate (specifically PE) ownership on quality of care and the types of procedures patients receive in these 2 specialties.^{3,5,6} Type 1 deals, in which a supergroup had already built scale and centralized management before receiving capital, may attenuate the effect of external investment on care delivery, whereas type 2 and type 3 deals, in which corporate entities exert greater control over which practices to acquire and how they operate, may produce larger effects. Pooling these transactions can therefore yield muted average effects.

These strategies may also differ in their regulatory visibility. Type 1 acquisitions of established supergroups may be more likely to exceed the transaction-value thresholds that trigger premerger notification under the Hart–Scott–Rodino Act,⁵⁴ whereas type 3 growth proceeds through numerous small acquisitions that each individually fall below the threshold and can eventually lead to an increase in market concentration.⁵⁵ We also find that type 1 chains often start as supergroups that gain local market share by acquiring other practices before receiving financial backing, raising the question of whether external capital drives consolidation or whether physicians consolidate in anticipation of a future sale. How these supergroups are organized, as meaningfully integrated or as looser financial arrangements assembled to raise prices via joint negotiation, determines both their antitrust treatment and speaks to an ongoing debate on whether they sustain competition by allowing physicians to remain in independent practice or reduce it by accelerating consolidation.^{56–58}

Although most research on corporate and PE growth in health care has focused on acquisitions,^{3,5,8,25} we document a second channel: organic entry. An acquisition largely reflects an owner’s decision to sell, whereas organic entry reflects individual physicians’ decisions to join corporate practices. Such decisions are consistent with documented pressures on independent practice—inadequate payment, rising costs, and administrative and regulatory burden—and with the particularly steep decline in ownership among physicians younger than 45.^{17,59} The growing importance of organic entry may also reflect corporate chains’ decision to pursue growth by attracting physicians instead of via acquisition, a channel that expands their footprint without generating the reportable transaction that can trigger antitrust merger review.⁵⁵ As such, corporate chains may be investing in

ways to attract both practicing physicians and new graduates, including better work environments or more competitive compensation packages.⁶⁰ Finally, physicians who choose corporate practice may differ systematically from those whose practices are acquired, meaning the channel of entry may shape the consequences of corporate ownership for clinical care.

Lastly, the observed trends in dermatology and gastroenterology may also provide insights into the ownership trajectories of other specialties. Dermatology is office-based and composed of many small, independent practices; it offers limited facility revenue for hospitals but presents corporate entities with scalable targets for roll-up and, where the landscape is fragmented, room to build chains de novo. This is consistent with the pattern we observed in dermatology, where corporate chains were the fastest-growing ownership category while hospital ownership grew more modestly, and with documented PE entry into other office-based specialties such as ophthalmology.^{24,61} Gastroenterology, by contrast, is procedure- and facility-based and therefore more financially attractive for hospital integration given the site-of-care payment differentials which benefit the hospital.^{12,62} This is consistent with the hospital-driven consolidation we observed in gastroenterology, where hospital ownership dominated in terms of both levels and growth. Other facility-based specialties, such as cardiology, have likewise seen substantial hospital integration⁶³ and more recently corporate acquisition activity,⁶⁴ suggesting that gastroenterology and cardiology may be following similar consolidation trajectories. While our study focuses on dermatology and gastroenterology, other specialties share similar structural features and face the same forces reshaping ownership across medicine. The approach we develop provides a means to track these changes—and assess their consequences—across other specialties.

Conclusion

This study provides a comprehensive characterization of the physician practice ownership landscape in dermatology and gastroenterology from 2012 to 2022. In dermatology, corporate chains were the fastest-growing ownership category, representing 16% of dermatologists by 2022. In gastroenterology, both corporate chains and hospitals represented growing ownership categories, accounting for 11% and 47% of gastroenterologists by 2022, respectively. Much of this growth occurred through acquisitions of existing supergroups, though corporate chains also grew through de novo chain creation. Distinguishing these archetypes can help future work assess whether effects typically attributed to PE vary by the acquiring chain's growth strategy. Similarly, the rise of organic physician entry raises the question of whether voluntarily joining a corporate chain affects physician behavior differently than being acquired. Overall, the taxonomy and data infrastructure we developed are a necessary first step toward understanding how different ownership types influence physician behavior.

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Supplementary material

Supplementary material is available at [Health Affairs Scholar](https://healthaffairscholar.org/) online.

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Conflicts of interest

Please see ICMJE form(s) for author conflicts of interest. These have been provided as [supplementary materials](#).

Data availability

The data underlying this article were provided by the Centers for Medicare & Medicaid Services (CMS) under a data use agreement (DUA). These data cannot be shared publicly due to the terms of the DUA. Researchers interested in obtaining access may establish a DUA directly with CMS. The code used to generate the analytic data and all output is publicly available at <https://github.com/physician-ownership/mdppas-ownership>.

Notes

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